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/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 18 of 117



A 76-year old man is brought to the hospital by emergency medical personnel after multiple shocks from his implantable cardioverter-defibrillator and syncope. His medical history is notable for diabetes, hypertension, and coronary artery disease status-post coronary artery bypass grafting eight years ago. He has an ischemic cardiomyopathy with left ventricular ejection fraction 20%. He is a current, every-day smoker.

On arrival, heart rate is 110 bpm, blood pressure is 80/60 mm Hg, and oxygen saturation 99% on room air. His jugular venous pulsations are visible at the angle of the jaw when sitting upright, lungs are clear and cardiac exam reveals tachycardia without murmurs. He has an S3. His legs are cool but without edema.

He is started on an amiodarone infusion and taken emergently to the cardiac catheterization lab where coronary angiography reveals a patent left internal mammary artery graft to the left anterior descending artery with known occlusion of his prior saphenous vein grafts. His native vessels are not revascularizable. Right heart catheterization reveals right atrial pressure 15 mm Hg, pulmonary artery pressure 63/35 mm Hg with pulmonary capillary wedge pressure 30 mm Hg. Cardiac index by Fick is 1.4. An intra-aortic balloon pump is placed in the right femoral artery and the patient is admitted to the coronary care unit

Initial laboratory results are notable for a sodium of 121 mmol/L, creatinine of 2.5 mg/dL, alanine transaminase of 345 U/L and aspartate transaminase of 512 U/L with arterial lactate of 5 mmol/L.

Which of the following is the next best step to manage this patient?

- A. Implantation of a durable left ventricular assist device
-  B. Administration of continuous inotropes
- C. Initiation of veno arterial extracorporeal membrane oxygenation
-  D. Enrollment in a palliative care program

Testing Point: The Board-certified AHFTC specialist should recognize when advanced therapies including LVAD or heart transplantation are futile in patients with end-stage heart failure.

Answer: D. Enrollment in a palliative care program

This patient has end-stage heart failure secondary to ischemic cardiomyopathy. He has presented with ventricular tachycardia storm with multiple shocks from his ICD. He has poor hemodynamics and no coronary lesion to be revascularized.

Answer A: Additionally, he has evidence of multi-organ failure which preclude him from candidacy for durable left ventricular assist device. Answer C: Given the lack of an advanced therapy option, escalation of support to extracorporeal membrane oxygenation (ECMO) would be unlikely to meaningfully change his outcome given his advanced age and comorbidities.

Answer B: Administration of continuous inotropes in some studies has been associated with improvement in quality of life but overall data are lacking. Additionally, inotropes are pro-arrhythmogenic and likely to worsen his ventricular tachycardia.

Therefore, the best option would be to discuss goals of care and engage palliative care.

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